

Paracetamol — Clinical Reference Guide

This document provides clinical reference information for healthcare professionals regarding the use, dosage, side effects, and contraindications of Paracetamol (also known as Acetaminophen). Always consult current guidelines before prescribing.

Dosage Guidelines

The standard adult dose of Paracetamol is 500mg to 1000mg per dose. The maximum recommended dose is 4000mg per 24-hour period. Doses should be spaced at least 4 to 6 hours apart. Do not exceed four doses in any 24-hour period. For elderly patients, a reduced maximum of 3000mg per day is recommended. Always use the lowest effective dose for the shortest duration necessary.

Paediatric Dosage

For children, dosing is weight-based. The recommended dose is 15mg per kilogram of body weight, given every 4 to 6 hours as needed. The maximum daily dose for children must not exceed 60mg per kilogram per day. Paracetamol is not recommended for infants under 2 months of age without medical supervision. Always use a calibrated measuring device when administering liquid formulations to children.

Side Effects and Adverse Reactions

Paracetamol is generally well tolerated when used at recommended doses. However, the following adverse reactions have been reported in clinical literature.

Common Side Effects

Most patients experience no significant side effects at therapeutic doses. Mild nausea has been reported in a small percentage of patients. Skin rash or allergic reactions are rare but possible. Patients with known hypersensitivity to Paracetamol should avoid this medication entirely.

Serious Side Effects

Hepatotoxicity (liver damage) is the most serious adverse effect associated with Paracetamol. This typically occurs following overdose or prolonged use above the recommended dose. Symptoms of liver damage include jaundice, abdominal pain, and abnormal liver function tests. Acute liver failure has been reported in cases of intentional or accidental overdose. Renal impairment has been documented in rare cases of chronic high-dose use. Immediate medical attention is required if overdose is suspected.

Contraindications and Drug Interactions

Before prescribing Paracetamol, clinicians must review the patient's full medication list and medical history for potential contraindications and interactions.

Absolute Contraindications

Paracetamol is absolutely contraindicated in patients with known severe hepatic impairment or active liver disease. Patients with a history of Paracetamol hypersensitivity must not receive this medication. Chronic alcohol use disorder significantly increases the risk of hepatotoxicity and is considered a relative contraindication requiring careful clinical judgment.

Drug Interactions Table

Drug	Interaction	Clinical Action
Warfarin	Enhanced anticoagulant effect	Monitor INR closely
Alcohol	Increased hepatotoxicity risk	Avoid or reduce dose
Carbamazepine	Reduced efficacy of Paracetamol	Consider alternative
Isoniazid	Increased liver toxicity	Use with caution
Metoclopramide	Faster Paracetamol absorption	Monitor for toxicity

Storage and Handling

Proper storage of Paracetamol is essential to maintain its efficacy and safety profile.

Storage Conditions

Store Paracetamol at room temperature between 15 and 30 degrees Celsius. Keep away from direct sunlight, heat, and moisture. Do not store in the bathroom or near a kitchen sink. Keep all medications out of reach of children and pets. Liquid formulations should be stored upright and used within the period stated on the label.

Overdose Management

Paracetamol overdose is a medical emergency requiring immediate intervention. Even if the patient appears asymptomatic in the first 24 hours, serious liver damage may develop. N-acetylcysteine (NAC) is the antidote for Paracetamol overdose and is most effective when administered within 8 to 10 hours of ingestion. Blood Paracetamol levels should be measured at 4 hours post-ingestion to guide treatment. Contact the National Poisons Centre immediately if overdose is suspected.

Patient Counselling Points

Always inform patients to read the label carefully before use. Remind patients never to take more than the recommended dose. Advise patients to avoid taking other products containing Paracetamol simultaneously. Patients should consult a pharmacist or doctor if symptoms persist beyond 3 days. Alcohol consumption should be minimised while taking Paracetamol. Inform

patients to seek immediate medical help if they accidentally take too much.